

Therapy

Therapeutic Options Recommended in Dermatitis Herpetiformis (DH)

Main option:

- Lifelong gluten-free diet (GFD) $\pm$ dapsone

Additional options (symptomatic):

- Sulfasalazine
- Potent topical corticosteroids
- Antihistamines

Therapeutic Decisions

A lifelong gluten-free diet, with or without dapsone, is the cornerstone of treatment for DH. The only causative therapy is a strict GFD, which must be implemented in every case. However, initiation of the GFD should occur **only after all diagnostic evaluations are complete**, including serologic testing and skin biopsy for direct immunofluorescence (DIF).

All other therapeutic options are **symptomatic** and target skin lesions and pruritus. They do not affect the underlying disease process or intestinal involvement and therefore **must not replace** the GFD. These may be considered under the following circumstances:

1. Symptoms are intolerable
2. Skin involvement is severe
3. Patient refuses or is unable to adhere to a GFD
4. Disease does not respond to a correctly implemented GFD

Dapsone is the most effective symptomatic treatment. Clinical improvement is typically seen within **3–4 days** of initiation, and this rapid response can support the diagnosis of DH.

Alternative symptomatic therapies—such as sulfasalazine, potent topical corticosteroids, and antihistamines—are significantly less effective. These should be considered only if dapsone is contraindicated, not tolerated, or refused by the patient.

The therapies mentioned above will be discussed in detail in the following sections.

Gluten-Free Diet (GFD)

The gluten-free diet is the sole causative treatment for DH. It involves the complete exclusion of gluten-containing cereals:

- Wheat
- Rye
- Barley
- Oats (unless specifically labeled as gluten-free, due to cross-contamination risks)

These can be replaced with naturally gluten-free sources of complex carbohydrates, such as:

- Rice
- Corn
- Pseudo-cereals: quinoa, millet, sorghum
- Flours derived from almond, chestnut, coconut, poppy seed, pumpkin seed, or sesame

Other safe foods include:

- Vegetables
- Fruits
- Legumes
- Dairy products (milk, cheese)
- Eggs
- Meat and fish (provided there is no gluten contamination during processing or storage)

Critical Note: Patients must **not start a gluten-free diet before diagnosis is confirmed**, as dietary withdrawal of gluten can lead to

normalization of serologic markers and histologic findings, thereby complicating accurate diagnosis.

Regulatory standards for labeling foods as "gluten-free" were established by **Commission Regulation (EC) No 41/2009**, which defined criteria for gluten content (≤ 20 ppm). An updated framework for consumer product labeling was implemented by the European Commission starting **20 July 2016**, improving transparency and safety for patients following a GFD.